4

The politics of patronage

THE articulation of the moral treatment, as concept and as practice, took place at a crossroads in the history of medical organization in France. The Law of 19 ventôse Year XI was putting the final seal on the destruction of the medical corporations at just the moment when Pinel was growing in celebrity, gaining adherents as the author of the *Traité*, and beginning to quantify his therapeutic experiments at the Salpêtrière. The convergence of these two trends poses the question of early psychiatric organization: What sort of community of specialist practitioners originally grew up around the paradigm of the moral treatment? Clearly the absence of the corporation would powerfully affect the shape of any such community. And while other, nontraditional modes of "sociability"[1] might develop in place of the corporate one, helping to create a sense of solidarity and collective identity among the new specialists, the psychiatric community would probably be to some degree constituted by the state. Like French medicine as a whole after 1803, it would probably have a highly significant point of attachment to the bureaucracy.

The organizational structure of a "patron" and a "circle" fits these specifications. It has been suggested that this informal structure – in which the so-called patron holds an important post or academic chair, and a small group of his students and associates (the "circle") share and develop his intellectual beliefs and receive his support in the advancement of their careers – is the key to the sociology of modern French intellectual life within the system of higher education.[2] Accord-

[1] Maurice Agulhon has made this term a fundamental category of the historical analysis of associational life. See his *La sociabilité méridionale: Confréries et associations dans la vie collective en Provence orientale à la fin du 18e siècle*, 2 vols. (Aix-en-Provence: Travaux et mémoires de la Faculté des lettres, 1966), esp. Vol. 1, pp. 7–15; and, more relevant in this context, *Le cercle dans la France bourgeoise, 1810–1848: Etude d'une mutation de sociabilité* (Paris: A. Colin, 1977).

[2] Terry N. Clark and Priscilla P. Clark, "Le patron et son cercle: Clef de l'université française," *Revue française de sociologie* 12 (1971): 19–39: and Terry N. Clark, *Prophets and Patrons: The French University and the Emergence of the Social Sciences* (Cambridge, Mass.: Harvard University Press, 1973), Ch. 2. The latter (pp. 67 and 67 n3) opts in English for the social scientific term "cluster" but indicates that a variety of other terms,

ing to this view, the patron derives his authority – and, more tangibly, his patronage – from his place in the bureaucratized state university. The state presence, though usually only hovering in the background of the circle, is nonetheless essential to its operation. At the same time, the patron possesses a large area of personal freedom and discretion; and this unmonitored aspect of the system helps account for the fact that both greatly gifted aspirants and those of decided mediocrity have typically succeeded in moving up the ranks of French higher education.[3] This structure is applicable to the proto-organization of French psychiatry as well. Like an academic discipline, the new psychiatric specialty developed within the matrix of the system of higher education – in its case, at the Ecole de médecine de Paris (called once again, after 1808, the faculty) and especially in the Paris hospice system, crucially linked to the faculty through the post-Revolutionary commitment to clinical instruction. It also attached itself firmly to a second part of the state bureaucracy, that of the Ministry of the Interior. By the proto-organization of psychiatry, I mean that purely informal and officially unrecognized organization that began to take shape in the opening decade of the nineteenth century and sufficed until the founding of the first formal organization of French psychiatrists, the Société médico-psychologique, in 1852. Since, however, the formal Société did not even pretend to assume all the functions of the informal patron-and-circle structure, the latter persisted after 1852 in tandem with its formal counterpart.

During the period under consideration in this chapter, French psychiatry was organized in rudimentary fashion in two circles that were sequentially, indeed genealogically related – the circle of Pinel, and the larger and more self-consciously specialist circle of his student Esquirol. The careers of these two men overlapped significantly. Although Esquirol was almost thirty years younger than his mentor, the latter, due to his period of self-assigned additional study at Montpellier and his long moratorium on the fringes of the medical world in pre-Revolutionary Paris, had been a notably late starter. He was almost fifty years old when he obtained his Bicêtre post in 1793 and embarked upon his most creative and important work. Thus it behooved Esquirol, whose filiopiety was intense,[4] to wait patiently in the wings. Not until 1817, when his mentor was in his seventies and he himself was forty-five, did he seem to have felt it permissible to take the kinds of initiative

including "circle," are actually used by the participants. Contemporaries usually referred to what I have here called the "circles" of Pinel and Esquirol as their "schools" (*écoles*).

[3] Clark and Clark stress this point as the fundamental explanatory value of their model; see "Patron et son cercle," pp. 19–20.

[4] See the testimony in Camille Bouchet, *Quelques mots sur Esquirol* (Nantes: Impr. de C. Mellinet, 1841), p. 2.

that would upstage Pinel. In that year, Esquirol began a course of clinical instruction in *maladies mentales* at the Salpêtrière, not part of the official curriculum at the faculty but still the first course in France to be devoted solely to that subject. Only then did he begin deliberately to acquire students of his own and to bring his own circle into existence.

The Pinel circle and the Esquirol circle were quite different organizational entities, a result both of changing historical circumstances and of the different personalities and goals of their patrons. Of the two men, Esquirol exploited the potential of the patron-and-circle structure much more fully. At his hands, the structure's statist base was accentuated, while at the same time the sense of community within the circle was intensively cultivated. The combination of these two strategies eventually enabled the Esquirol circle to go some distance toward the professionalization of the new medicine of the insane, a process in which – given the peculiarly French nineteenth-century concept of "profession" – collaboration with and legitimation by the state would prove paramount. In this chapter, I will describe the origins and the modes of sociability of the two circles. Woven into the account will be an analysis of some of the professionalizing efforts of the Esquirol circle; but this latter subject, too large to exhaust here, will continue to figure in subsequent chapters.

The Pinel circle

Pinel's circle was not made up exclusively of psychiatric students, for Pinel himself wore two hats as a physician. He was as much a nosographer, attempting to reconstruct the whole classificatory system of disease in line with Idéologue principles of "analysis" and clinical observation, as he was a healer of the insane. Pinel's position as patron derived indirectly from two official posts: the chair in medical pathology he had held at the Ecole de médecine in Paris since 1795, and dating from the same year, his appointment as chief physician of the Salpêtrière. On the basis of these two posts, he devised a third function for himself. Recognizing that the Ecole's abstract commitment to clinical instruction far exceeded the actual provision that had been made for it, Pinel, on his own initiative, opened a private clinical course at the Salpêtrière, receiving extra tuition from the students who chose to attend.[5] Based in the infirmary of the hospice, where the full panoply of diseases could be observed, the *cours particulier* had a simple format. The medical history of a newly arrived patient was taken "by one of my more advanced and experienced students and . . .

[5] Mireille Wiriot, *L'enseignement clinique dans les hôpitaux de Paris entre 1794 et 1848* (Paris: Thèse de médecine, 1970), pp. 51–52. It was eighteenth-century custom for professors at the medical faculty to supplement their official lectures with private courses, for which students paid them special fees.

read aloud at the bedside. During this reading, I fix the attention of the [other] students on the features that can be regarded as specific to the disease under consideration, and then I assign the disease to the place it occupies within my nosographic framework. In certain doubtful cases, I discuss the greater or lesser value or the equivocal character of certain signs, and sometimes I suspend my judgment until the disease is further along in its course."[6] These clinical lessons in what was called medical semiotics or "the science of signs" – that is, in "reading" the sick body for purposes of diagnosis and prognosis – were extraordinarily popular among Parisian medical students during the era of the Consulate and Empire. Entailing much closer professor–student contact than official lectures from the podium at the Ecole, they were responsible for the formation of a circle of students around Pinel.

In the area of clinical instruction, still so relatively novel and so attractive to the student body, Pinel had only one rival at this time: Jean-Nicolas Corvisart, an expert in heart disease who occupied the sole official clinical chair established by the Ecole de médecine in 1794, that at the Charité hospital.[7] The popularity of Corvisart always seemed self-explanatory to his contemporaries – he had a forceful personality and a brilliant oratorical style – while that of Pinel always seemed to require explanation. For Pinel was timid and retiring, his teaching marred by a speech impediment. "He could not say two words without hiccuping," recalled one student, who nonetheless found himself in the Year X rushing "to bring my three louis to Monsieur Pinel for the privilege of attending his visit to the old women of the Salpêtrière." Devoid of the external trappings of pedagogical charisma, Pinel came to be highly valued and sought after by students for the pure content of his teaching. They cited his "great penetration as a clinician" and "great clarity as a professor." "It was he," said one, "who made me a physician."[8]

The cohesiveness of the Pinel circle was of a particular and limited sort. Although it is possible that the members were invited to gatherings in Pinel's rustic country home in Torfou, not far from Paris,[9]

[6] Pinel, *La médicine clinique rendue plus précise et plus exacte par l'application de l'analyse*, 3d ed. (Paris: Brosson, 1815), p. ii.

[7] See Wiriot, *Enseignement clinique*, pp. 47, 51. Clavareau, *Mémoires sur les hôpitaux civils de Paris* (Paris: Impr. de Prault, Year XIII/1805), pp. 103–4, indicates that Corvisart began clinical instruction at the Charité before the Revolution, in 1787. The gentlemanly Pinel–Corvisart "rivalry" is discussed by Leuret, "M. Esquirol," *Gazette médicale de Paris*, 2d ser., 9 (January 2, 1841): 1; and by E. Pariset, "Eloge de Ph. Pinel," *Histoire des membres de l'Académie royale de médecine* (Paris: J.-B. Baillière, 1845), Vol. 1, pp. 246–49.

[8] P.-B. Bailly, quoted in Wiriot, *Enseignement clinique*, p. 52 (for the first and last quotes); and Leuret, "M. Esquirol," p. 1.

[9] See I. Bricheteau, *Discours sur Philippe Pinel, son école et l'influence qu'elle a exercée en médecine* (Paris: Impr. de C. L. F. Panckoucke, 1828), pp. 17–18; and René Semelaigne, *Les grands aliénistes français* (Paris: G. Steinheil, 1894), p. 219.

gathering was not a central motif. The circle was constituted primarily through each member's tie with Pinel rather than through members' face-to-face relationships with one another. Pinel forged the ties, definitively claimed the students as his own, by his help in fostering their careers; and the kind of help he offered bore the strong imprint of his personal style.

The most favored students among those who came to the clinical course were treated in the usual patronal manner: Gainful employment was obtained for them. Thus, at Pinel's express request to the Paris hospice administration, Landré-Beauvais was made his assistant (*premier adjoint*) at the Salpêtrière in 1801 and was later promoted to the salaried position of "*médecin ordinaire* specially charged with indigent inmates." Esquirol was made "*médecin ordinaire* of the section for the insane" at the same institution in 1811 when, following the death of Pussin, Pinel requested a replacement for his trusted concierge in the form of a specialist – or, as he put it, "a physician . . . devoted exclusively to the study of insanity" – and argued that Esquirol, with his many years of *maison de santé* experience, was the only man around suited for the job.[10] Pinel dispensed to his students the other, unofficial patronage at his disposal. Thus Landré-Beauvais took over Pinel's private clinical course at the Salpêtrière in 1810, when handling both that assignment and his pathology lectures at the faculty become too arduous for the aging *maître*.[11] On Isidore Bricheteau, a former Salpêtrière intern who would later write a shrewd account of the modus operandi of the Pinel circle, Pinel bestowed the commission to prepare "in collaboration with him or, if you will, under his patronage" numerous articles for the sixty-volume *Dictionaire des sciences médicales*.[12] A full decade in the making, the *Dictionaire* was the project of the adventurously capitalistic publishing firm of Panckoucke, which had earlier masterminded the scheme for the *Encyclopédie méthodique*.[13] Pinel was part of the regular stable of authors for both these compendia.

On at least one occasion Pinel also used his own modest fortune to aid the career of a student. To enable Esquirol to undertake the intensive study of insanity in an appropriate milieu, he reportedly put up the security for the house and garden that Esquirol rented on the rue de Buffon in 1801 or 1802, and which he converted into a private *maison*

[10] Monique Dumas, *Etienne Esquirol: Sa famille, ses origines, ses années de formation* (Toulouse: Thèse de médecine, 1971), p. 108.

[11] Wiriot, *Enseignement clinique*, p. 53.

[12] Bricheteau's account of the circle is in his *Discours sur Pinel;* on Bricheteau himself, and for the phrase quoted here, see C. Sachaile de la Barre, *Les médecins de Paris jugés par leurs oeuvres* (Paris: Chez l'auteur, 1845), p. 147.

[13] On Panckoucke, see Robert Darnton, *The Business of Enlightenment: A Publishing History of the Encyclopédie, 1775–1800* (Cambridge, Mass.: Harvard University Press, 1979). It is Panckoucke *fils* who published the *Dictionaire;* Panckoucke *père* is the protagonist of Darnton's story.

de santé for lunatics.[14] Pinel then proceeded not only to refer patients to Esquirol's new *maison* but also to support the venture through discreet "advertisements" within his own widely-circulated texts. Already in 1803 the second edition of the *Nosographie philosophique* was touting "the commendable establishment . . . directed by Citizen Esquirol, one of my students especially concerned with the treatment of mania and with putting into practice the principles I developed in my *Traité de l'aliénation mentale.*"[15] The opening pages of the second edition of the *Traité* announced that, in addition to detailing the author's own treatment procedures at the Salpêtrière, "I have also set forth the happy results that have been obtained for the past several years in an analogous establishment (that of Dr. Esquirol) founded upon the same principles – that is, observation and experience." Later in that book, Pinel described Esquirol's *maison* in some detail – its lush vegetation, pleasant accommodations for patients and their domestic servants, and the attractively humane and efficacious use of such aspects of the moral treatment as musical soirées in "a vast salon" for convalescent patients.[16] By 1810, Esquirol's *maison* was well on the road to success. The *Annuaire médical de Paris* ranked it as one of the three best *maisons de santé* for lunatics in the capital, noting that it was well-known throughout France, visited by savants from abroad, and (no doubt a causal factor in all this) that it had been "justly esteemed by Professor Pinel in his new work on insanity."[17]

But, it must be stressed, these most-favored students were exceptional. In general Pinel did not make such concerted efforts to find suitable employment for the members of his circle. This failing was particularly evident by contrast to the very efficient placement system Corvisart had developed for his students. Recalling this fundamental difference between the two "rival" clinical professors, Bricheteau attributed Pinel's characteristic posture of "not worrying about [his students'] material advancement" to his deep-rooted disdain for questing after social status and esteem, his kind of otherworldly devotion to science. Drawing his pedagogical prescriptions from his autobiography, Pinel "wanted his students to devote a great number of years to study before undertaking the practice of medicine; he constantly distanced his private pupils from this latter goal. Thus several of them with merit and reputation lived in a condition of extreme poverty bordering on distress." Even later in life, Pinel denied himself many of the material rewards of his successful scientific career, choosing to live with his wife and children in the very modest apartment provided for him at the

[14] Bouchet, *Quelques mots*, p. 3.
[15] Cited in Dumas, *Esquirol*, p. 89.
[16] Pinel, *Traité médico-philosophique sur l'aliénation, ou la manie*, 2d ed. (Paris: Brosson, 1809), pp. ii, 369–70 n1, 373.
[17] Cited in Dumas, *Esquirol*, p. 92.

Salpêtrière, having "no equipage other than a hackney-cab," indulging only in the purchase of the quite unpretentious Torfou property.[18] Corvisart, on the other hand, apparently experienced no conflict between science and worldly success, and he went about skillfully furnishing his students with "the means of reaping [from their talent and learning] a just profit for science and for themselves by obtaining for them, on the strength of his credit, *positions* and places." Corvisart's wholehearted assumption of this aspect of the patronal role, Pinel's reluctant and half-hearted one, seemed to Bricheteau to contain the seeds of the future: It explained the longevity of Corvisart's influence in French medicine while that of Pinel, since it had not generated a network of students in institutional strongholds, "began to dwindle from the moment that the *maître* ceased to be able to exert himself personally (*payer de sa personne*)."[19] Pinel's timidity, so often remarked upon by contemporaries, no doubt also helped to determine his choice of patronage style. He must have preferred to avoid the bureaucratic politics of place hunting just as he preferred to avoid, even when attacked, polemical disputes over medical doctrine. In both cases he rationalized his avoidance by construing such activities as motivated by worldly ambition rather than disinterested scientific curiosity and therefore as unworthy of his attention.[20]

The patronage that Pinel did typically offer the members of his circle took the form of highly visible printed praise. Just as he commended Esquirol's *maison* in the pages of his *Nosographie* and *Traité*, so would he "encourage those setting out on a career by generously citing their works" in his own – "a sort of honor after which everyone [in the circle] strove and which was almost always a happy augury" for a debutant.[21] Thus a remark in the *Traité* about the importance of identifying diseases by unequivocal manifest signs is accompanied by the following footnote: "I have always put great store by the semiology of disease, and I have observed with pleasure that Monsieur Landré-

[18] Bricheteau, *Discours sur Pinel*, pp. 13, 13 n1, 17. Cf. the lifestyle of Etienne Pariset, who while a physician at Bicêtre and later the Salpêtrière maintained two apartments, one at the hospice and another in the center of Paris. George D. Sussman, "Etienne Pariset: A Medical Career in Government under the Restoration," *Journal of the History of Medicine* 26 (1971): 60–61.

[19] Bricheteau, *Discours sur Pinel*, pp. 13–14, italics in the original. Personal experience underlay Bricheteau's comments. As Sachaile de la Barre noted, "Monsieur Bricheteau remained without a hospital appointment for a very long time," but apparently modeling himself on Pinel, he also remained unembittered, caring only for "science" and "smiling with pity" at those physicians who sought "fame and fortune." *Médicins de Paris*, p. 148.

[20] With respect to polemics, Pinel also forbade his students to reply for him, even after the particularly violent attack on his classificatory scheme by Broussais in 1816. Bricheteau, *Discours sur Pinel*, pp. 15 and 15 n1.

[21] Ibid., p. 13.

Beauvais made it a special object of his researches. I have no doubt that his *Traité des signes des maladies* (one volume in-octavo, Paris, 1809, at Brosson, bookseller) will have all the success that it has the right to expect." To a discussion in the *Médecine clinique* of a chemical analysis of the Salpêtrière water supply conducted by a certain Schwilgué, Pinel appends a footnote informing the reader that "Since my first edition, a premature death has carried off, in the midst of his labors, Monsieur Schwilgué, whom I number among my most distinguished pupils. He published an excellent treatise on materia medica." A passing comment in the *Nosographie philosophique* informs the reader that Schwilgué's treatise was based upon years of careful study "at the Salpêtrière hospice of the action of pharmacological preparations on the living body" and adds that Pinel consulted the treatise "incessantly" for the treatment of disease.[22] Less effusive but nonetheless patently supportive references to the books, articles, and dissertations of his students lard Pinel's *Nosographie*.[23]

The system had its pros and cons. "All the works published by the pupils of Pinel lent one another a mutual support," observed Bricheteau, "and the assent of the *maître* covered all of them with the protection of eminence." But Bricheteau was also aware that Pinel's broad discretionary powers as patron – even in this clearly limited sense of patronage – did not always foster scientific excellence. The affectionate kindness which Pinel felt indiscriminately for most of his students and which motivated his laudatory citations of their efforts "sometimes had the disadvantage of exalting mediocrity"; within the Pinel circle's collective corpus, "the strong works sustained and propped up the weak ones."[24] Furthermore, through his strategy of citation Pinel created his circle in a reified fashion, as a corpus of scientific texts whose authors shared a common identity element rather than as a lively association of interacting physicians.

All of Pinel's students, whether or not their main interest was in the treatment of the insane, were kept abreast of the latest advances in that area. Pinel's *Nosographie philosophique*, the breviary of the post-1803 generation of French medical students,[25] contained discussions of mania and melancholia that summarized its author's specialized researches and presented his considered opinions. But only two of Pinel's students devoted themselves single-mindedly to the new medicine of the insane – Esquirol and Guillaume Ferrus. Already the holder of a Paris

[22] Pinel, *Traité*, 2d ed., p. iii n1; *Médecine clinique*, 3d ed., p. viii n1; *Nosographie philosophique, ou la méthode de l'analyse appliquée à la médecine*, 6th ed. (Paris: Brosson, 1816), Vol. 1, p. lxxxviii.

[23] See, e.g., *Nosographie philosophique*, 6th ed., Vol. 3, pp. 55 n1, 59, 75, 117 n1.

[24] Bricheteau, *Discours sur Pinel*, p. 13; Pinel's sympathetic relationship with his students is also described in Dupuytren, "Pinel," *Journal des débats*, November 7, 1826.

[25] Bricheteau, *Discours sur Pinel*, pp. 8–9.

medical degree by 1804, Ferrus entered Pinel's orbit only in 1814, upon reentering civilian life after serving for a decade as a military surgeon in the Napoleonic army. Pinel immediately brought him into the Salpêtrière as a humble *adjoint;* not until 1819 was Ferrus put on the payroll of that institution with the title and low salary of a *médecin suppléant.*[26] Indeed, Pinel's repeated requests to the Paris hospice administration failed to obtain a full-fledged post for him until 1826, when shortly before Pinel's death, Ferrus was made chief physician of the insane at Bicêtre.[27] Esquirol thus had a good fifteen years' head start on Ferrus, both in the study of insanity and as Pinel's protégé. There could be no doubt that, in things psychiatric, Pinel's mantle would fall to him.

The Esquirol circle

Almost as soon as Esquirol appeared at Pinel's private clinical course at the Salpêtrière around 1799 (he had already been enrolled in Pinel's official course at the Ecole de médecine for two years),[28] a deep bond formed between these two men of the Midi – "paternal attachment on the part of Pinel, truly filial love on the part of Esquirol," according to one source close to Esquirol.[29] But whatever fundamental emotional affinity they shared, they had significantly different social-class backgrounds, and these were reflected in the different styles of patronage they adopted.

While Pinel's father had been a small-town physician in the region of Toulouse, Esquirol's was a wealthy wholesale cloth-merchant in Toulouse itself, belonging, like his father before him, to the most prosperous and highly esteemed *corps* in that provincial capital. Commercial prominence in eighteenth-century Toulouse went hand in hand with prominence in public life: Esquirol *père* had been elected to the Bourse des marchands, a law court that decided commercial disputes and whose judgments, if appealed, went directly to the Parlement. On the eve of the Revolution, he was elected to the *capitoulat,* the traditional governing corporation of the municipality only recently opened to wholesale merchants, membership on which carried the automatic privilege of ennoblement. Esquirol hailed, then, from the uppermost stratum of the bourgeoisie, from that place in the social hierarchy where the bourgeoisie was on the verge of transformation into the robe nobility.

[26] AAP: Procès-verbaux du Conseil général des hospices civils de Paris (henceforth CGH), Liasse 58, f. 251. Ferrus received 1200 francs per year as opposed to Esquirol's 2000 francs as *médecin ordinaire;* see AAP: Salpêtrière 2K6.

[27] Sachaile de la Barre, *Médecins de Paris,* pp. 288–89; Semelaigne, *Grands aliénistes,* p. 219.

[28] See Dumas, *Esquirol,* pp. 73–74.

[29] Leuret, "M. Esquirol," p. 2.

Like any good Old Regime bourgeois with aspirations to *vivre noblement*, Esquirol *père* owned land outside the city. But his personality seems to have been marked not by the insouciant elegance of a would-be noble but by an austerity and strong sense of civic responsibility probably typical of the Toulouse merchant corps. He served for at least a decade, without remuneration, as one of the directors of the Grave, the *hôpital-général* in Toulouse; this post, which involved time-consuming visits of inspection, was usually filled by members of the affluent commercial bourgeoisie of the city. He is reported to have used his own fortune to buy grain for the populace of Toulouse during a period of scarcity in 1789 when popular riots threatened. The two-story residence in a central square of Toulouse in which Esquirol grew up was a typical Toulousain merchant dwelling: the commercial establishment on the first floor and the family apartments above it, "the whole decor giving an impression of severity."[30] This portrait of Esquirol *père* is important here because, as a patron, his son replicated its central features: a sense of high social standing, which would put Esquirol at ease in his crucial dealings with men in power, coupled with a deeply ingrained seriousness and a sense of obligation to those – be they lunatics or unemployed students – who could not fend for themselves.

The emergence of Esquirol as patron, 1817–1819. Esquirol launched himself as a patron from a less secure official base than had Pinel: He was not a professor at the Paris faculty (nor was he ever to become one); he was not, at the outset, the chief physician of a Paris hospice but only a *médecin ordinaire.* What he did have was a clearer vision than his mentor of what he wished to accomplish through the medium of organization. Hence, while Pinel's role as patron was generally characterized by influence unused and opportunities unrealized, Esquirol's was marked by an active but always tasteful creation of opportunities. In particular, the aspiring patron was able to ingratiate himself to great effect with the state bureaucracy.

For several years beginning in 1817, Esquirol laid the foundation for his new patronal role. In the first place, the clinical course in *maladies mentales* he initiated in the makeshift quarters of the Salpêtrière dining hall[31] proved a great success. Clinical instruction had proliferated at the Paris faculty since the days when Pinel and Corvisart had monopolized the field; noting that "the capital is very rich in *écoles de clinique,*" a medical student handbook of 1818 advised that "the student ought not accord an exclusive preference to one rather than another [but] frequent them by turns." Nonetheless Esquirol was, a medical gossip columnist reported, one of the clinical instructors to whose hospital

[30] Dumas, *Esquirol,* pp. 21–43.
[31] Bouchet, *Quelques mots,* p. 4.

visits "students flock with a kind of frenzy."[32] But Esquirol did not stop there: Providing instruction in *maladies mentales* was only one element in a larger design. In 1810, 1814, and again in 1817 he had embarked, on his own initiative and at his own expense, on a series of systematic tours of the facilities for lunatics throughout France.[33] These fact-finding missions eventuated in two publications: a memoir presented to the minister of the interior in September 1818 and published six months later, and a much more extensive and detailed version of it, the 1818 article "Maison d'aliénés" in the *Dictionaire des sciences médicales.*[34] Together these texts comprised the basic program or manifesto of the new medical specialty which Esquirol envisioned and which he would gear his circle toward implementing.

That program, directed both to the medical community, by means of the *Dictionaire* article, and to the lay political world, by means of the often sentimental *Mémoire* ("It is impossible not to be moved almost to tears while traversing the country where Monsieur Esquirol paints for us the miseries of the insane," said a contemporary reviewer),[35] consisted of four interrelated main points. The first and overarching one was that the medicine of the insane was a specialty. Esquirol did not say this in so many words, but he said as much: Insanity should be treated in *hôpitaux spéciaux* by physicians with special training, those who had "fruitfully cultivated this branch of the healing art" and had renounced all other clientele.[36] A careful argument was mounted in support of these propositions. If lunatics continued to be housed in *hôpitaux-généraux* and *dépôts de mendicité,* as Esquirol had discovered the vast majority of them presently were, they would invariably continue to suffer neglect; for the entire staff in such institutions – physicians,

[32] J.-P. Maygrier, *Guide de l'étudiant en médecine,* 2d ed. (Paris: Gabon, 1818), p. 101; Véridique Alleyes-Allears, "Lettres médicales sur Paris," *Revue médicale historique et philosophique* 2 (1820): 194.

[33] The dates of the first two tours are given in the nearly contemporaneous article, "Esquirol," *Dictionaire des sciences médicales: Biographie médicale,* Vol. 4 (Paris: C. L. F. Panckoucke, 1821) p. 58. Esquirol requested and received permission for a three-week leave of absence from the Salpêtrière in August 1817 "to visit the hospices of the North, having seen those of the Midi and the West of France." AAP: CGH 48, ff. 384–85. Both Leuret, "Esquirol," p. 4, and Dr. Gérard Marchant, *Esquirol: Discours prononcé à la séance solennelle de la Société impériale de médecine . . . de Toulouse* (Toulouse: Douladoure, 1868), p. 13, testify that the tours were self-financed.

[34] Esquirol, *Des établissements des aliénés en France et des moyens d'améliorer le sort de ces infortunés. Mémoire présenté à son excellence le Ministre de l'Intérieur en septembre 1818* (Paris: Impr. de Mme Huzard, 1819); and Esquirol, "Maisons d'aliénés," *Dict. des sci. med.,* Vol. 30 (1818): 47–95. When he reprinted the *Mémoire* twenty years later, Esquirol prefaced it with the remark that it had been "written at the request of the Minister of the Interior," but this may well have been retrospective embroidery; see his *Des maladies mentales considérées sous les rapports médical, hygiénique et médico-légal* (Paris: Baillière, 1838), Vol. 2, p. 398.

[35] Unsigned, *Gazette de santé,* April 21, 1819, p. 375.

[36] Esquirol, *Mémoire,* pp. 23, 27; "Maisons," p. 62.

directors, nurses, orderlies – gave their attention by natural preference to the other inmates in the heterogeneous population, finding all other inmates easier and more pleasant to deal with than lunatics. The physicians had an additional reason for avoiding rather than succoring the lunatics in their charge. Their utter ignorance of any cure for insanity had left them profoundly "discouraged" – hence the need for a *médecin spécial* with special expertise.[37] Esquirol's subsequent success should not blind us to the fact that his tactics here were humble, suppliant, and even contained a touch of "charlatanistic" showmanship. In financing a tour of France which furnished supporting evidence for a proposal to the Ministry of the Interior that a specialized medical facility be established, Esquirol was operating much like the oculist Forlenze a decade or so before.

The second point might be characterized as the primacy of Paris. Esquirol took as axiomatic that all expertise in France about the treatment of lunatics was to be found in Paris and that any improvement in treatment procedures must necessarily take the form of an exportation of that expertise from the Parisian center to the benighted periphery. In his memoir and again in the very first sentence of his dictionary article, he described the motivation of his tours as "wanting to assess the influence in the rest of France of the ameliorations introduced in the lunatic institutions of Paris."[38] He found that influence quite weak indeed. Only in provincial capitals had "Paris furnished the example"; elsewhere "the most revolting barbarism" still prevailed. Lunatics, Esquirol believed, were best removed from "local influences," from those "prejudices which, in many provinces, make [them] regarded as incurables."[39] So deep was Esquirol's Paris-centrism that he even felt compelled to cast belated aspersions on the medal presented to the administrator of the Avignon lunatic hospice by the Lycée des Arts in 1797. "That was a little comedy performed to gladden the hearts of the patriots of the Midi," he said cuttingly. "That hospice never enjoyed . . . a reputation that would justify such a distinction. [Furthermore] when I visited [it], everything appeared to me contrary to the success of the treatment of the insane."[40] There was, then, a perfect congruence between Esquirol's belief that any reform of the care of the insane had, for scientific reasons, to emanate from Paris, and his choice of the central state bureaucracy as the agency of that reform.

The third point in Esquirol's program was that institution building was urgently necessary for the development of the new medicine of insanity. "A lunatic hospital is an instrument of cure," he asserted laconically, thus fully integrating the internal therapeutic aspect of this

[37] Esquirol, *Mémoire*, pp. 12, 20, 22–23.
[38] *Mémoire*, p. 7; "Maisons," p. 47.
[39] *Mémoire*, pp. 7, 26.
[40] "Maisons," pp. 51–52.

new medicine with its external institutional aspect. Architectural features such as courtyards for promenades, separate wards for different varieties of madness, and ground floor lodgings for furious lunatics, where security could be achieved with a minimum of lugubrious bars and padlocks, would make specially constructed hospitals devoted exclusively to the insane markedly more therapeutic than the existent quarters within *hôpitaux-généraux* and *dépôts de mendicité*. Esquirol therefore proposed to the minister of the interior the construction of a network of twenty such "special public establishments" dispersed in a uniform manner throughout France and superimposed upon the network of royal law courts so that each lunatic hospital would be affiliated with a neighboring *cour royale*. The minister of the interior would appoint the directors and doctors of each such hospital and oversee and coordinate the whole network; each hospital would serve the population of several departments and the relevant prefects would participate in its administration.[41]

Esquirol's fourth point was the definitive medicalization of the care of the insane, a point already evident in the writings of Pinel but now made by his disciple much more forcefully and with much more overt preoccupation with issues of professional status. The lunatic hospital was to be, as Esquirol stipulated in a revealing passage, the domain of a grandiose and specifically medical authority:

> The physician must be, in some manner, the vital principle of a lunatic hospital. It is he who should set everything in motion; he should regularize all actions, just as he has been called upon to be the regulator of all thoughts. . . . The action of the administration, which governs the material aspect of the establishment and supervises all the employees, ought to be hidden. Never should the administration appeal a decision made by the physician, never should it interpose itself between the physician and the lunatics or between the physician and the non-medical staff (*les serviteurs*). The physician should be invested with an authority from which no one is exempt.

Continuing in much the same vein, Esquirol sought "to justify the necessity of this influence" by reference to its curative efficacy. He cited autobiographical example. He exercised, he had found, more influence over the lower-class madwomen of the Salpêtrière than over the wealthy patients in his private sanatorium: "The inmates of [the former] establishment regard me as being of a station much superior to their own; thus it has several times happened that I have returned a lunatic to reason, as if by enchantment, simply by granting her an interview in my office; some of these have given signs of cure that very instant." Esquirol is alluding here to the somewhat magical, authoritar-

[41] *Mémoire*, pp. 30, 38–41. The one "voluntaristic" aspect of the scheme is that money for building be raised by a public subscription, undertaken by the state bureaucracy.

ian component which had always been part of the Pinellian moral treatment.[42] But he now drew significant practical inferences from the indispensability of that component. The healing power residing in the person of the physician – the so-called enchantment of medical authority – might either be maximized or left to languish; and recognition of that fact entailed an obligation on the part of the "local authorities." They could not afford to be neutral bystanders but must treat the physician of lunatics "with distinction"; "thus will be prepared the influence that [the physician] should exercise over the individuals admitted to his hospice."[43] Esquirol's demand, in other words, was that lunatics be entrusted to physicians and that the physician of lunatics be accorded, with the aid of local notables and bureaucrats, a high and honorable place in the larger community.

Like Pinel before him, but again more insistently, Esquirol looked to linguistic change as an additional vehicle of medicalization, as a way to demarcate a specifically medical domain and to indicate the placement of insanity within it. Pinel had spoken of the desirability of purging medicine of everyday language, of the "jargon circulating in the commerce of civil life"; he had urged the substitution of the word *aliénation* for the vulgar *folie*.[44] Now, to signify a rite of passage – the formal transfer of insanity from the lay to the medical purview and the concomitant discarding of the traditional lay prejudices about its incurability – Esquirol suggested that a new name be affixed to the institution housing lunatics: "I would like these establishments to be given a specific name which presents no painful idea to the mind; I would like them to be called asylums." And he added, "Those who know how much influence words have over men will not be at all astonished at the importance I attach to these little details."[45] Conversely, he was during his tours of the provinces always alert to linguistic localisms that suggested the backward, premedical mentality it was his self-appointed mission to eradicate. "Angers is the first city where I heard the name *exilés* given to insane people (*aliénés*)."[46]

[42] The magical curative authority of the gifted *aliéniste* continued to be part of Parisian psychiatric rhetoric well into the nineteenth century. While an intern at Bicêtre, Paul Broca (who would later found French physical anthropology) wrote home to his mother regarding the chief physician Leuret: "I, like everyone else, admired the power of fascination (*pouvoir fascinateur*) he exercises over our madmen." Paul Broca, *Correspondance*, Vol. 1 (Paris: Typographie Paul Schmidt, 1886), April 4, 1845, p. 281.

[43] "Maisons," p. 84.

[44] *Nosographie philosophique*, 1st ed., Vol. 1 (Paris: Crapelet, Year VI), p. 1; and *Traité*, 2d ed., pp. iii, 128–29.

[45] *Mémoire*, p. 26.

[46] Quoted in Jacques G. Petit, "Folie, langage, pouvoirs en Maine-et-Loire (1800–1840)," *Revue d'histoire moderne et contemporaine* 27 (1980): 557.

The institutional direction in which Esquirol had pointed the new specialty in 1818 was confirmed in an affair at the Paris Faculty of Medicine that same year. By a decree of December 4, the Commission de l'Instruction publique, the five-man board that governed the Université de France under the Restoration, created at the Paris medical faculty a chair in "special pathology relating to mental diseases," giving as its rationale that it wanted to carry out the "original plan" of 1794 by "filling all the chairs which ought to compose" such an institution of higher learning and carefully "earmarking" the chairs so that they covered all "the branches of learning most appropriate to the current needs of medical science." The decree specified that the new and unprecedented chair of mental diseases would be created through the transformation of one of the two existent chairs of legal medicine – at that moment vacant – and that it would be filled either by a transfer from within the professoriate or by a new appointment.[47] When, four days later, this decree was read as a fait accompli before an extraordinary session of the faculty, Pinel and his colleague Duméril, "believ[ing] themselves duty-bound to protest against this act," immediately deposited their written objections on the table. To cope with "the difficult questions raised by this incident," the faculty decided to appoint an ad hoc committee to study it. Chosen on the spot by secret ballot, the committee included the clearly partisan Pinel.[48]

Reporting back to the assembled faculty two months later, the committee recommended that both chairs of legal medicine be retained but that one of the two professors be charged with "giving a course in mental medicine considered principally in relation to the public institutions devoted to the treatment of insanity." There was precedent for this sort of piggyback curricular strategy: A course in the history of medicine was at that time attached to the chair of legal medicine. A few days later, the Commission de l'Instruction publique declared itself amenable to these "somewhat tardy observations" of the faculty, which they conceded "tend to prove that a chair of pathology will ill attain the goal of the Commission and that it would be more fitting to attach this new instruction to legal medicine than to pathology." A new decree of February 23, 1819 modified the decree of December 4, 1818 accordingly.[49]

What can be made of this curious story in which Pinel, who had spent the previous two decades laying the groundwork for a cogent medicine of insanity, rushes forward to block the creation of a chair in that very subject? Since the supporting documents mentioned in the

[47] AN: F^{17*} 1764, Procès-verbaux de la Commission de l'Instruction publique, December 4, 1818, f. 556.

[48] AN: AJ^{16} 6232, Procès-verbaux . . . de la Faculté de médecine de Paris, December 8, 1818, f. 721.

[49] Ibid., February 13, 1819, f. 90; February 18, 1819, ff. 91–92.

faculty minutes are missing, we can only speculate about Pinel's motives. The president of the Commission de l'Instruction publique and the signatory of the original December 4 decree was Pierre-Paul Royer-Collard, the Doctrinaire political philosopher and cohort of Guizot.[50] His brother was Antoine-Athanase Royer-Collard, the chief physician of Charenton since 1806 and since 1816 the incumbent of a chair in legal medicine. Indeed, A.-A. Royer-Collard had been the very first appointment to the Paris medical faculty made by the newly created commission under P.-P. Royer-Collard after the more democratic practice, devised by the Revolution, of selection by *concours* had been abandoned by the Restoration government.[51] Did Pinel believe that he saw the handwriting on the wall – that A.-A. Royer-Collard would be transferred to the new chair in mental pathology – and did he seek to prevent such an outcome, if only by demoting the chair to a course?

Such a hypothesis is entirely plausible, for all the evidence points to a discreetly conducted but bitter rivalry between the Pinel circle and A.-A. Royer-Collard. The latter was, after all, an ambitious man who headed one of the few special facilities for lunatics in France and who had, through family connections, an independent "power base" in medical politics. He had never studied with Pinel and had no affective ties to him.[52] He had published virtually nothing about *aliénation mentale,* so that his high place in the nascent medical specialty could hardly have seemed warranted by his scientific contributions. And he remained aloof, collaborating with Pinel in their area of supposed common scientific interest only when such collaboration was mandated, for limited purposes, by a court of law or by the police.[53] The rivalry was no doubt fueled when Pinel made an attempt to have Esquirol ap-

[50] P.-P. Royer-Collard had also been the chief advocate of the formation of the Commission de l'Instruction publique, a Restoration divergence from the bureaucratic structure of the Napoleonic Université. See Louis Liard, *L'enseignement supérieur en France, 1789–1893* (Paris: A. Colin, 1894), Vol. 2, pp. 125–47.

[51] That the abandonment of the *concours* had smoothed the way for the aid of the medical Royer-Collard brother by the political one did not escape contemporary critics; see Rouzet, "Pétition addressée aux deux Chambres sur la nécessité de rétablir le concours pour l'obtention des chaires vacantes dans la Faculté de Médecine," *Revue médicale historique et philosophique* 2 (March 1820): 59–73.

[52] See his *Essai sur l'aménorrhée, ou suppression du flux menstruel* (Paris: Gabon, Year X/1802), p. 66 n1, where he identifies Hallé as "mon illustre maître."

[53] One such collaboration is described at length by Royer-Collard in *Observations sur un écrit ayant pour titre Mémoire pour Mme de Chambon, appelante du jugement qui nomme M. Fréteau administrateur provisoire de la personne de Mlle d'Arbouville* (Paris: Impr. de Vincard, 1806), AN: AJ^2 100, and it could not have endeared Royer-Collard to Pinel. The two physicians had been named by a Paris tribunal as jointly responsible for treating a certain Mlle d'Arbouville for insanity. The situation became an imbroglio; and after Royer-Collard complained repeatedly about the conduct of Pinel's trusted coworker Pussin, Pinel washed his hands of the whole business and resigned. For another collaboration, see Maxime Laignel-Lavastine and Jean Vinchon, "Pinel médico-legiste," *Ann. m.-p.*, 12th ser., 2 (1927): 67.

pointed to Charenton in 1806 after the death of Gastaldy,[54] and the post was captured instead by Royer-Collard. Later Royer-Collard became the butt of jokes in the pages of a new medical journal which Esquirol had helped to found, depicted as a beneficiary of nepotism, a man whose rewards were incommensurate with either his efforts or his achievements. It was said that a petition for meritocratic selection of professors at the medical faculty had been quashed for his sake; that his lectures on legal medicine were shamelessly cribbed from Fodéré's treatise on the subject; that he shirked his responsibilities at Charenton by visiting patients only three days a week.[55]

Esquirol, who was not a member of the faculty, did apply for the vacant chair of legal medicine, no doubt hoping to teach the new course in mental medicine; but his candidacy was unsuccessful.[56] And when the chair was won by the toxicologist Orfila, the assignment for the mental medicine course devolved, logically enough, upon the other professor of legal medicine, A.-A. Royer-Collard.[57] If Pinel's tactics were intended to keep official instruction in *maladies mentales* as the prerogative of his own circle, they had only minimal efficiency. But on another level, the tactics also aimed to assert a fundamental social and institutional orientation in the new medical specialty: Mental medicine, the emended decree implied, should not be taught narrowly in a pathology course, but in a broadly contextual manner as a part of legal medicine, and with particular emphasis on the public facilities for the care of the insane. Through his protest, Pinel was thus supporting and extending the message of Esquirol's memoir and dictionary article. Indeed, Esquirol had in the former alluded to the connection between mental medicine and legal medicine, suggesting that his proposed asylum network be superimposed on the network of royal courts in part because of the legal problems surrounding the civil status and institutionalization of lunatics.[58] In the long run, the creation of this course in *médecine mentale* was of little practical significance, for the punitive closing of the Paris Faculty of Medicine by the Restoration government in 1822 eradicated the new instruction entirely. But for our purposes, the episode is additional evidence of the efforts made by Esquirol – acting here largely through his cooperative mentor Pinel – to enforce a particu-

[54] See E. Szapiro, "Pinel et Esquirol: Quelques commentaires sur les débuts d'une amitié," *Ann. m.-p.*, 134 (June 1976): 59–61.

[55] Alleyes-Allears, "Lettres médicales," pp. 178–79, 195. On Esquirol's role in founding the *Revue médicale historique et philosophique*, see Semelaigne, *Pionniers*, Vol. 1, p. 194.

[56] See the details of the balloting in AN: AJ[16] 6232, February 23, 1819, ff. 94–95.

[57] The standard secondary sources usually state, erroneously, that A.-A. Royer-Collard received a "chair" in mental medicine; see, e.g., Semelaigne, *Pionniers*, Vol. 1, p. 109. The *Revue médicale* later discussed the forthcoming course with predictable sarcasm: "In the past year, Dr. Royer-Collard . . . has already, he says, prepared *six lessons* of this course, so all hope is not lost." Alleyes-Allears, "Lettres médicales," p. 201.

[58] Esquirol, *Mémoire*, p. 38.

lar definition of the emergent specialty of mental medicine as he prepared to assume the role of psychiatric patron.

Esquirol's tours of the lunatic institutions of France had the effect, which he had no doubt hoped for, of providing him with an entrée into the upper echelons of the bureaucracy. He presented his memoir to the minister of the interior in September 1818; barely a month later he had been "discovered" by Edouard Laffon de Ladébat, chief of the ministry's bureau of assistance and hospitals and a career functionary whose interest in wresting lunatic institutions from local jurisdiction and welding them into a new and uniform state system dated back at least to 1813.[59] Laffon de Ladébat had found something of a soul mate in Esquirol. After hearing about the physician, he hastened to establish personal contact with him and was soon making copious use of Esquirol's data and recommendations in support of his own. In a long memorandum to the minister of the interior in October 1818, he observed that the Conseils généraux of the departments had lately been voting funds for the enlargement of existing facilities for lunatics in *hôpitaux-généraux* and *dépôts de mendicité;* he believed that this wrong-headed trend must be halted and financial investment made instead in new "special establishments." To this end he proposed the formation of a commission composed of an architect, a physician and an administrator to design a model lunatic institution that could be reproduced throughout provincial France. Four months later this proposal was again addressed to the minister of the interior, this time by Guizot, the newly appointed director-general of departmental and communal administration. Guizot's version of the proposal, which was ultimately accepted, expanded the size of the commission to include three physicians and two administrators; thus it was that Esquirol and A.-A. Royer-Collard served on it side by side.[60]

The commission did get as far as drawing up the preliminary blueprint for the model institution; it printed a limited edition of its descriptive brochure, distributing the copies to those administrators and physicians deemed most qualified to suggest revisions.[61] And in July 1819,

[59] See G. Bollotte, "Les projets d'assistance aux malades mentaux sous la Restauration," *Ann. m.-p.* 124 (1966): 388–402, esp. 386 for Laffon de Ladébat's letter of September 9, 1813 to the minister of the interior arguing for "a certain number of central establishments" for lunatics; for the full text of that letter, see AN: F^{15} 444.

[60] See Bollotte, "Projets d'assistance," pp. 390–93, 396 for the texts of Laffon de Ladébat's memorandum, with its numerous direct quotations from and paraphrases of Esquirol, and Guizot's memorandum.

[61] AN: F^{15} 1892, letter from the vice president of the Conseil général des hôpitaux et hospices de Paris to the minister of the interior, April 23, 1822. The brochure is *Programme d'un hôpital consacré au traitement de l'aliénation mentale pour cinq cents malades des deux sexes* (Paris: Impr. de Mme Huzard, 1821); a copy can be found in AN: F^{15} 1892. It was reprinted in 1824 in slightly altered form, with B. Desportes given as author; see BN: Te^{66} 183.

Minister of the Interior Decazes issued a circular to the prefects calling for a suspension of all current projects for the enlargement of lunatic quarters in mixed facilities and the diversion of funds voted for this purpose to the construction (through the collaboration of several departments) of special *maisons centrales.* The circular bore the mark of Esquirol's influence and rhetoric, for it stipulated a daily medical visit at all lunatic establishments and observed, "It would be good to invest [the physician] with great authority."[62] Over the next few years, Laffon de Ladébat pressed for implementation of the circular and haggled with the prefects over the details.[63]

But these efforts gradually petered out, the result of the demise, in 1820, of the liberal Decazes ministry which had nurtured them. The commission, for example, "ceased meeting in 1821."[64] Just as Laffon de Ladébat's ideas had met with a relatively cool reception under the previous right-centrist ministry – Minister of the Interior Lainé had thought them important enough to mention in his 1818 report to the King, but he had counseled against using government monies to build special *maisons centrales* for lunatics and recommended passively waiting for private benefactors to endow them[65] – so they failed utterly to elicit ministerial favor during the period of almost uninterrupted right-wing ascendancy from 1820 until the end of the Restoration.[66] From Esquirol's own perspective, however, the gains of the short-lived *maison centrale* experiment had been substantial. He had made solid bureaucratic contacts – witness his appointment by the Ministry of the Interior to the lucrative post of inspector of the university in 1823[67] – and these would stand him in good stead during the next two decades.

[62] Circular of July 16, 1819 in *Circulaires, instructions et autres actes émanés du Ministère de l'Intérieure,* Vol. 3 (Paris: Impr. royale, 1821), pp. 487–91. In the *Mémoire* (pp. 27–28), Esquirol had stated his preference for medical visits several times a day and had made clear the unacceptability of any frequency less than once a day.

[63] See, e.g., AN: F^{15} 143, letter of the prefect of the Charente to the Bureau des secours et hôpitaux of the minister of the interior, July 24, 1822.

[64] This is Esquirol's testimony in the revised and expanded version of "Maisons d'aliénés" in *Maladies mentales,* Vol. 2 (henceforth "Maisons d'aliénés," 1838 ed.), p. 448.

[65] See Lainé, *Rapport au Roi sur la situation des hospices, des enfans trouvés, des aliénés, de la mendicité et des prisons* (Paris: Impr. royale, 1818), pp. 18–19.

[66] See Bollotte, "Projets d'assistance," p. 397 on Laffon de Ladébat's fruitless efforts to revive the *maison centrale* project in 1822 and 1828.

[67] Semelaigne, *Grands aliénistes,* p. 138. A.-A. Royer-Collard had held this post for many years and received an annual salary of 6000 francs for it, twice as much as his professorial salary; for the latter, see AN: F^{17} 2381. All aspects of public instruction were under the authority of the Ministry of the Interior from 1792 until 1824; see Jean Vial, "L'administration centrale de l'instruction publique en France de 1792 à 1855," *Paedagogica Historica* 9 (1969): esp. pp. 120–25.

The workings of the circle. Esquirol acquired a student following during the years 1817–25, when he taught his clinical course at the Salpêtrière. In 1825, A.-A. Royer-Collard died, thus removing from the scene the one man who might have become a rival psychiatric patron and leaving vacant the post of chief physician at Charenton, which was this time promptly offered to Esquirol. At Charenton, where he remained until his death in 1840, Esquirol did not continue his clinical course; probably the distance of that "suburban" institution from the medical faculty on the Left Bank seemed a decisive deterrent to student attendance. But since the regulations of Charenton provided for four *élèves internes* and an indeterminant number of *élèves externes,* he had not fully renounced his pedagogical role.[68] The membership of any informal group is difficult to determine with precision; but if three criteria are posited for membership in the Esquirol circle – training under Esquirol by a person either pursuing a medical degree (the usual case) or already in possession of one; a subsequent career in *médecine mentale;* and public self-identification or identification by others as an Esquirol pupil – a core circle of some nineteen members can be readily named. The vast majority among this group were born in the period 1794–1801 and encountered Esquirol in their twenties at the Salpêtrière; only two were products of Esquirol's later Charenton phase. The circle was thus a virtually complete and functioning entity before Esquirol left his Salpêtrière post. Almost all of its members were provincials who had migrated to Paris for medical training; with time they would be transformed into the bearers of a quintessentially "Parisian" medicine. Almost all were sons of the bourgeoisie, representing the full spectrum of the early nineteenth-century middle class – from coupon-clipping landowners, wholesale merchants, and wealthy lawyers, at the one end, to urban craftsmen and bakers at the other. A few diverged from this typical social background: Three came from peasant milieus; one, surely the most colorful of the lot, was born illegitimate and without officially known paternity into the old Norman sword nobility. (See Appendix.)

The core members have left vivid descriptions of the customs of the circle. Trélat and Brierre de Boismont remembered

> The Sunday luncheons where the disciples, mingled with other, already distinguished men, were invited, by the cordial affability of the *maître,* to take part in the most elevated discussions of the physiological and morbid phenomena of the mental realm.[69]

At these luncheons, Esquirol would typically "indicate the areas of the science which had [hitherto] been only superficially touched" and

[68] Semelaigne, *Grands aliénistes,* p. 132; Esquirol, "Mémoire historique et statistique sur la maison royale de Charenton," *Ann. d'hyg. pub.* 13 (1835): 111–12.

[69] Ulysse Trélat, "Notice sur François Leuret," *Ann. d'hyg. pub.* 45 (1851): 247.

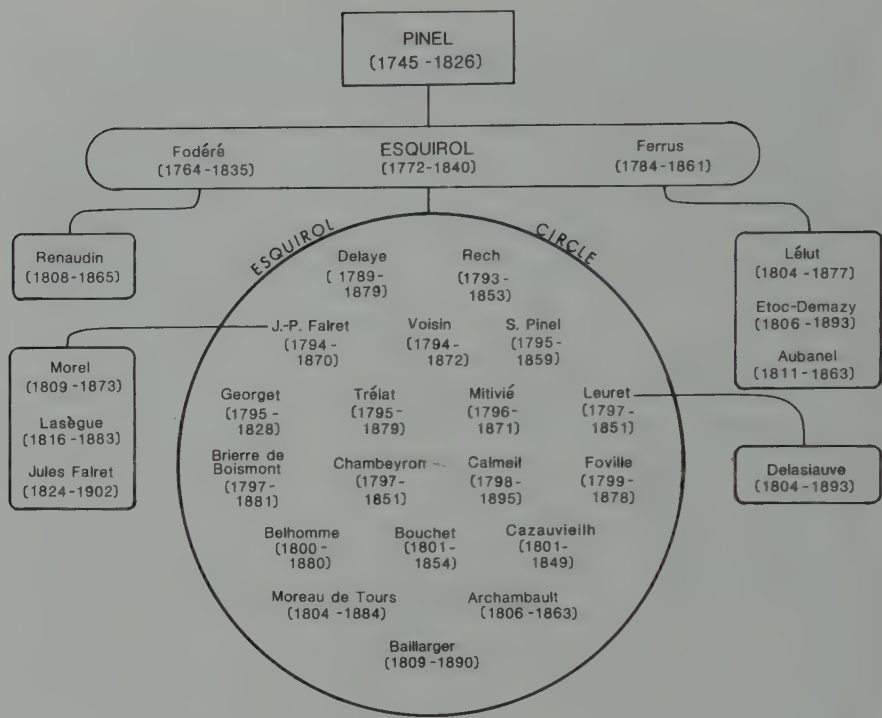

Figure 1. The "genealogy" of French psychiatry in the early nineteenth century: teachers and students

would thus put his students on the track of new and fruitful research.[70] The paternal ambiance that suffused these gatherings was uppermost in Bouchet's recollection of them. Esquirol, who was married but had no children, "loved to be surrounded by [his pupils], to unite them into a single family; . . . each Sunday saw seated at his table ten or twelve young physicians . . . , all joined by an equal tenderness for their *maître* and all regarded by him with the same paternal affection."[71] The students whose talents most impressed Esquirol he urged to compete for the annual prize – 300 francs and a copy of Pinel's *Traité* – which he had founded in the pivotal year 1818 to honor the best study of any aspect of insanity.[72] By means of this prize, he sought

[70] A. Brierre de Boismont, "Notice biographique sur M. F. Leuret," *Ann. m.-p.*, 2d ser., 3 (1851): 514.

[71] Bouchet, *Quelques mots*, p. 12. Some of Moreau de Tour's memories of this formative experience were, appropriate to a "familial" milieu, gustatorial; as an aging *aliéniste* at the Salpêtrière in the early 1880s, he used to say, "On déjeunait fort bien chez M. Esquirol." Semelaigne, *Pionniers*, Vol. 1, pp. 299–300.

[72] See Trélat, "Notice sur Leuret," p. 247; Bouchet, *Quelques mots*, pp. 5–6; and Semelaigne, *Grands aliénistes*, pp. 131–32.

"to excite [in his students] that spirit of competition (*émulation*) which elicits labor by supplying it with an immediate goal"; and, according to one student, the lure of the prize succeeded both in prompting individuals to undertake the study of insanity and in leading them to embark upon those significant projects that later made their reputations.[73] The prize contributed as well to the sense of cohesiveness and collective endeavor of the circle, since Esquirol, creating a kind of small-scale academy, chose the panel of judges from among the auditors of his course.[74]

Unlike Pinel, who usually shrank from the patronal function of finding positions and places for his students, Esquirol emphasized that function. Much more committed than Pinel to founding a specialty, he could not afford his circle to be evanescent and knew that to insure its continuity, he must give it institutional roots. The patronage most readily at Esquirol's disposal was within his own *maison de santé*. In the late 1820s he had moved the *maison* from the rue de Buffon to a more spacious property in the village of Ivry outside Paris, building a brand new facility there in accordance with his own therapeutic-cum-architectural precepts. In both locations, the *maison* was a flourishing enterprise, catering to a highly affluent clientele; and it was another source of Esquirol's celebrity, turning him into something of a household word. As a Paris newspaper wrote of his *maison* in 1827:

> Who has not heard about the excellent treatment that the doctor gives to lunatics, the care and attention of which they are the object? But who does not also know that it is impossible to be admitted on the rue de Buffon for less than 10 or 15 francs a day? . . . To see madhouses (*maisons d'aliénés*) risen to such extraordinary prices, one would be tempted to believe that insanity is a privilege and that, without being a bureaucrat or a capitalist, it is inadvisable to rave. However, the janitor, the bricklayer, and the errand-boy are allowed to go crazy just like the others for, if equality is anywhere, it is assuredly in human misery. Ah well, what I wish for then is a low-cost asylum, an inexpensive Esquirol.[75]

Professional positions at the *maison* were not numerous, and they were reserved for the students with whom Esquirol was most intimate: his own nephew Mitivié; the brilliant Georget; and, after Georget's premature death, Leuret, the last two virtually surrogate sons. (Was it only a coincidence that the social origins of these two specially favored students were among the humblest of any members of the circle? Georget's father was a peasant farmer and miller of grain, Leuret's a baker. Esquirol's self-conception as patron may have had a tinge of noblesse oblige, making him particularly responsive to talented young men who

[73] Bouchet, *Quelques mots*, pp. 5–6.
[74] Semelaigne, *Grands aliénistes*, p. 132.
[75] "Des maisons de santé," *Journal de Paris*, August 6, 1827.

were his inferiors not only in age and medical experience but also in social standing.)

Within Paris and the surrounding area, there was not much else available for the circle. The Salpêtrière slot vacated by Esquirol went to Etienne Pariset, a physician without psychiatric training who had earned the gratitude of the government by participating in a mission to Spain to study the yellow fever epidemic. Pariset's old slot at Bicêtre went to Guillaume Ferrus, for whom Pinel had long solicited a senior appointment. The resourceful Esquirol did inveigle the administration to reestablish a position which had fallen into desuetude at Charenton – that of *médecin-inspecteur du service de santé* – and, since the position was within his purview as chief physician, he placed his student Calmeil in it.[76]

But Esquirol's most significant achievement as a patron was his placement of students in provincial lunatic institutions, thus actualizing one of the circle's central tenets – the dissemination of the new Parisian expertise in *maladies mentales*. The bureaucratic contacts that Esquirol had acquired in the Ministry of the Interior, especially during the interval 1819–1821, together with those he had acquired in local administration through repeated provincial visits both before and after 1819,[77] were decisive here; for, lacking an official position legitimating his actions, he nonetheless, as Trélat put it proudly and approvingly, "endowed the principal *maisons de traitement* of our country with his former students, the most capable to honor science and to serve humanity well."[78] There was nothing covert about Esquirol's exercise of unofficial influence. As one member of the Esquirol circle, Chambeyron, stated it baldly in a letter to the minister of the interior: "In 1835, Esquirol made me chief physician of the hospital for the insane at Rennes."[79] In a public speech, another member of the circle, Bouchet, described (though without mentioning his own name) how he had happened to become physician at the new lunatic institution of Nantes in 1834:

> During one of [Esquirol's] voyages, in the year 1819, he went to Nantes and was received by the Commission administrative des hospices in a distinguished manner which touched him deeply . . . and which he always remembered with gratitude. Thus when in 1825 [that Commission] submitted to him the initial plans for the Saint-Jacques lunatic asylum, he

[76] See Sussman, "Etienne Pariset," pp. 52–55; Esquirol, "Mémoire historique et statistique sur Charenton," pp. 109–10.

[77] In 1821 Esquirol received permission for a two-month holiday from the Salpêtrière, both to mend his health and to visit "all the lunatic houses of France once more" in order to assess recent improvements. AAP: CGH 72, ff. 153–54.

[78] Trélat, "Notice sur Leuret," pp. 247–48.

[79] AN: F^{15} 3915, letter dated May 18, 1848.

considered his careful examination of them as an acquittal of a debt. Later he himself proposed the physician for [the asylum] . . .[80]

Esquirol made comparable unabashed avowals. Of the new Saint-Yon lunatic institution at Rouen: "Much admired are the order and discipline introduced there from the beginning by Dr. Foville, a Salpêtrière student who was appointed chief physician of the asylum when, on the invitation of the prefect, I put his name forward."[81] Of the renovated lunatic facility of the Grave in Toulouse, where Esquirol's father had long ago been an overseer: "Dr. Delaye, who had been my student at the Salpêtrière and whom I pointed out to the administration, was named physician of the division of the insane."[82]

To these various posts the Esquirol students – missionaries in their own way – brought the enlightened methods vindicated in Paris. Bouchet stressed the "moral treatment" in Nantes, construing it to mean primarily the patient's resumption of work in his former occupation. Only work (as he reasoned in Pinellian vocabulary) could provide a sustained "distraction from delirious impressions and thoughts" while avoiding any dangerous bombardment of "the imagination with sensations which cause it to stray from real life."[83] Chambeyron testified that, at the Saint-Méen hospice in Rennes, he had done everything possible "to transform into a *maison de santé* what had really been merely a lunatic depot." At a meeting of the local society of savants, he displayed with a dramatic flourish the instruments used "to contain madmen" at Saint-Méen before his arrival and which he had promptly banned – "iron shackles, thumbscrews, a type of large bag in which was enclosed the head of a lunatic whose screams were troublesome and which was then attached to a wall or some other stable point." In their stead Chambeyron had introduced "the new method" of "hygienic and moral care" whose "better effects are incontestably demonstrated by the higher rate of cure."[84] And irritated that the local hospital administration had publicly reversed one of his decisions, he sternly lectured its members about the "lessons that I received" – obviously from Esquirol – "on the governance of lunatic houses." "Permit me to tell you, gentlemen, that if you desire the reform and prosperity of

80 Bouchet, *Quelques mots*, p. 7.

81 Esquirol, "Maisons d'aliénés," 1838 ed., p. 453. Esquirol's Rouennais connection was of long standing; in 1821 the Ministry of the Interior had sent him to Rouen to confer with local administrators about their institution for the insane, then in the planning stage; AAP: CGH 68, f. 495.

82 Esquirol, "Maisons d'aliénés," 1838 ed., p. 479.

83 Bouchet, "Compte moral du quartier des aliénés de l'hospice général de Nantes," *Ann. m.-p.*, 1st ser., 8 (1846): 405, 407.

84 ADIV: 18Td 1, Société des sciences et arts de Rennes, Session of June 15, 1840. I learned of this document in Jacques Léonard, *Les médicins de l'Ouest au XIXème siècle* (Lille: Atelier reproduction des thèses, 1978).

Saint-Méen, material amelioration will not suffice. You must also give to the medical chief whom you have appointed *an authority that, in the eyes of his subordinates, is entirely autocratic* and subject only to the direct but hidden control of the administration."[85]

In Montpellier at a newly constructed special quarter for the insane at the Saint-Esprit hospice, Dr. Rech, a native of the city who had trained under Esquirol during a three-year stay in Paris – and who had been chosen for the post in 1821 by a supervisory board of local notables committed to what Rech called "modern principles" – disseminated the influence of the *maître* by touting "the moral treatment," insisting upon the superiority of medical authority in the institution, and offering clinical lessions in *maladies mentales*. "Should that happy innovation be confined to the capital?" he mused about this new instruction.[86] (Rech identified himself and was identified by others as an Esquirol student, but Esquirol did not claim him as such. No doubt the young provincial's sojourn at the Salpêtrière made more of an impression on him than on his pedagogue.)[87]

In at least one instance local doctors balked at the imposition – so integral to Esquirol's plan – of "Parisian" medicine and medical personnel. The appointment of Chambeyron to the Saint-Méen hospice in Rennes brought forth a written protest signed by twenty-eight of the town's physicians. A "revolting injustice," they charged, had been perpetrated against their "honorable colleague" Dr. Macé, who had held the then part-time post of physician at the lunatic hospice for twelve years but had been summarily dismissed to make way for Chambeyron – not named but called here, significantly, "a stranger" – on the "ridiculous pretext" that the latter possessed "special knowledge." "The administration" had even had the gall to solicit applications for Macé's vacant post, a procedure later revealed to be a sham – and an "insult" to the two physicians of Rennes who did apply – because the "stranger" had already been appointed. The outrage of these physicians was at root a response to their entirely accurate perception that "the

[85] ADIV: Fonds de l'Asile Saint-Méen (as yet uncatalogued during my September 1982 visit to Rennes), letter of Chambeyron dated May 7, 1836, my italics.

[86] Rech, "Clinique de la maison des aliénés," *Ephémérides médicales de Montpellier* (June 1826): 120, and "Quelques considérations sur l'étude des aliénations mentales," *Nouvelles annales cliniques de la Société de médecine pratique de Montpellier* 1 (1822): 354. See also Colin Jones, "Treatment of the Insane in Eighteenth- and Early Nineteenth-Century Montpellier," *Medical History* 24 (1980): 385–87.

[87] Rech described himself as "the pupil and friend" of Esquirol in "Quelques considérations," p. 358 n1. Cf. Esquirol, "Maisons d'aliénés," 1838 ed., p. 455, which describes Rech only as "professor at the faculty of Montpellier," as opposed to descriptions of other young alienists as Salpêtrière pupils. But Marchant, once a Charenton intern under Esquirol, regarded Rech as a member of Esquirol's "school"; see his *Esquirol*, p. 12. So did the editorial in the first number of *Esculape: Journal des spécialités médico-chirurgicales* (June 1, 1839): 2.

administration," itself an emanation from Paris, disparaged local medical talent, preferred its Parisian equivalent, and was able to enforce that preference.[88]

When Esquirol was unable to place a student in a full-time position, he often resorted to a temporary palliative. One variety of the moral treatment he prescribed for some of his private patients was travel, a distracting "change of milieu"; and at such times he characteristically selected one of his students to accompany and care for the troubled voyager. Moreau de Tours, for example, began his career in this manner and even traveled to the Orient, where he undertook research on the psychological effects of hashish. Indeed, one of Esquirol's elegists compiled a list of some fourteen students for whom the *maître* had found temporary employment as glorified travel companions.[89] Penurious students were sometimes given lodging in Esquirol's own home: Georget stayed there for a full eight years.[90] Bouchet summed up matters by saying that Esquirol "never ceased helping his students . . . , facilitating their debuts in *le monde* with all his influence." When all else failed, the help came, with "unostentatious generosity" from their mentor's own purse.[91]

There was a significant difference between the way Esquirol conducted the circle internally and the face that it turned to the outside world. Internally, the intellectual independence of the students was respected, never sacrificed as the price that had to be paid for patronal support. Esquirol may have "reigned uncontested in the field of insanity" after 1826 (by which time death had removed both Pinel and A.-A. Royer-Collard), but he apparently did not reign autocratically over his students or strive to turn them into epigones; he was not one of "those princes of science who can tolerate only mediocrities around him."[92] "I will never forget," said one member of the circle, "that he gave me his first testimony of affection after a discussion in which we held conflicting views."[93] In its external relations, however, the circle presented a united front. "That medical phalanx," one of the members dubbed it retrospectively, referring to its solidarity in combat.[94] Its capabilities as a "phalanx" were most apparent in its defense of the diagnostic cate-

88 ADIV: 20 Md 3, Petition dated August 5, 1835. A note in Léonard, *Médicins de l'Ouest*, brought this document to my attention.

89 Semelaigne, *Pionniers*, Vol. 1, p. 295; Pariset, "Eloge de J.-E.-D. Esquirol," *Histoire des membres de l'Académie de médecine*, Vol. 2, Note A, p. 476.

90 Raige-Delorme, "Nécrologie: Georget," *Arch. gen. de med.* 17 (1828): 328; Leuret and Baillarger also lived with Esquirol; see Pariset, "Eloge," Note A, p. 476.

91 Bouchet, *Quelques mots*, p. 11.

92 Brierre de Boismont, "Notice sur Leuret," p. 514.

93 Bouchet, *Quelques mots*, p. 11.

94 Brierre de Boismont, "Notice biographique sur C. Bouchet," *Ann. m.-p.*, 2d ser., 6 (1854): 312. Clark, *Prophets and Patrons* (p. 67 n3) indicates that military names – "regiment," "militia" – are typically used for clusters by their members.

gory monomania, which became the subject of a long, heated controversy beginning in the mid-1820s. In this endeavor, the geographical dispersal of the circle through provincial appointments did not vitiate its force but enhanced it; for since cohesiveness had been maintained despite physical separation, the circle had acquired a national presence.[95] The monomania controversy and its larger significance, as well as the circle's role in it, will be explored in the next chapter. I will confine myself here to a discussion of another of the circle's "combat maneuvers," one which at the same time illustrates the way in which the organizational form of patron-and-circle might work to discourage excellence: the case of Antoine-Laurent Bayle.

Bayle is now recognized as a major theoretical innovator in French psychiatry. In the early 1820s he identified general paralysis as a discrete disease entity characterized by speech impairment, progressive weakening of motility in the arms and legs, and a delirium degenerating into dementia. General paralysis quickly caught on as a diagnostic category (or as Bayle put it with fine wit, "assumed the rank of bourgeoisie among disease families").[96] Less than two decades after it was identified, between one-ninth and one-sixth of the patients in Paris lunatic institutions were being diagnosed as its victims.[97] Yet Bayle was never able to obtain a position as a physician in a lunatic institution. The precise reasons for this professional failure are obscure, but by all accounts Esquirol was the crucial factor.[98] While a medical student in Paris, Bayle had attached himself to A.-A. Royer-Collard at Charenton, and it was there, in what he described as "one of the finest and most beneficial establishments devoted to the treatment of the insane" and with "bounties" constantly heaped upon him by Royer-Collard, that he had carried out his original researches on general paralysis on almost 200 patients.[99] When Royer-Collard died in 1825, Bayle was without a

[95] Not only had the identity "Esquirol student" been deeply impressed upon those placed in the provinces, but these emissaries seem also to have stayed in close touch with the Paris psychiatric scene. In 1839, for example, Delaye came back to Paris with Gérard Marchant, a former Esquirol intern who now assisted Delaye at the Grave lunatic hospice in Toulouse, and the two visited Bicêtre to observe Leuret's new therapeutic experiments. See Marchant, "Quelques mots sur l'influence thérapeutique de la musique exécutée par les aliénés," *Esculape*, October 15, 1840, pp. 85–86.

[96] Bayle, "De la cause organique de l'aliénation mentale accompagnée de paralysie générale," *Ann. m.-p.*, 3d ser., 1 (1855): 411.

[97] Baillarger, "Nouvelles considérations sur la paralysie générale incomplète," *Ann. m.-p.*, 1st ser., 8 (1846): 425.

[98] See Erwin H. Ackerknecht, *A Short History of Psychiatry*, trans. S. Wolff, 2d ed. (New York and London: Hafner, 1968), p. 51; Semelaigne, *Pionniers*, Vol. 1, pp. 244–49; Stefan Müller, *Antoine Laurent Bayle: sein grundlegender Beitrag zur Erforschung der progressive Paralyse* (Zurich: Juris, 1965), p. 19.

[99] A. L. J. Bayle, *Nouvelle doctrine des maladies mentales* (Paris: Gabon, 1825), p. 15. For lavish praise of Royer-Collard's intellect and a subtly competitive remark about the

protector, and Esquirol showed no inclination to take the talented young man, the protégé of his rival, under his wing. Instead Bayle found himself ousted from his adjunct position at Charenton as soon as Esquirol took charge of that institution, and he retreated to the assistant librarianship of the Paris medical faculty. Decades later, the Esquirol circle showed its mettle as a "phalanx": It banded together to vindicate the master's ostracism of Bayle by insisting that Bayle had no claim to the discovery of general paralysis, that the real priority belonged to one of their own number, Delaye.[100]

The dynamics of recruitment: specialization and the "doctor glut"

In 1824 Esquirol was pondering aloud the new popularity among young physicians of the study and treatment of insanity, a popularity manifested in the large enrollment of his Salpêtrière clinical course and the ease with which he had formed his circle. He had always recognized that tending lunatics was an intrinsically unattractive occupation requiring "a sort of self-abnegation."[101] Thus Esquirol opined in 1824 that the "happy efforts of medicine and the daily successes it obtains" since the Pinellian breakthrough had led "a greater number of physicians to turn to the study of *maladies mentales,* however painful that study may be."[102] This roughly Kuhnian analysis is surely accurate, but it is also incomplete: Not only internal factors – advances in medical theory and therapy – but also external ones involving professional demography must be taken into account to explain how psychiatry attracted its recruits in the crucial period of its formation.

The Restoration and July Monarchy were a period of medical overcrowding (or *encombrement,* as contemporaries called it), a surplus in the supply of doctors over the effective demand for their services – at least in the major cities, where doctors congregated by preference. The phenomenon was probably part of a broader one, extending to the liberal professions generally and touching all the countries of Western

scientific superiority of Charenton over Bicêtre and the Salpêtrière, see Bayle, *Traité des maladies du cerveau et de ses membranes* (Paris: Gabon, 1826), "Avant-Propos."

100 See the assertions in Baillarger, "Nouvelles considérations sur paralysie générale," p. 424, and Trélat, "De la paralysie générale," *Ann. m.-p.,* 3d ser., 1 (1855): 233. In 1855 Bayle defended his own claims to priority before the Académie de médecine; see his "De la cause organique," p. 410.

101 Esquirol, *Mémoire,* p. 27.

102 Esquirol, "Existe-t-il de nos jours un plus grand nombre de fous qu'ils n'en existait il y a quarante ans?" (1824), as reprinted in *Maladies mentales,* Vol. 2, p. 734. The Paris administrator Desportes offered a similar analysis in *Compte rendu au Conseil général des hospices et hôpitaux civils de Paris sur le service des aliénés . . . pendant les années 1822, 1823, et 1824* (Paris: Impr. de Mme Huzard, 1826), p. 25.

Europe.[103] It was described and bemoaned in the medical literature,[104] and acknowledged as a fact of social life in the fiction of the day. The narrator of Balzac's *Z. Marcas*, a law student living in a Left Bank garret in the 1830s with his medical student friend, offered a typically somber and ironic description of the situation:

> We thought only of amusing ourselves. . . . We could see no future in the two professions our parents had forced us to embrace. There are a hundred lawyers, a hundred doctors for every place. The crowd obstructs these two avenues . . . , which are really two arenas: one kills oneself there, one enters into combat not with steel or firearms but by intrigue and calumny, by horrible labors, by campaigns in the realm of the intellect as murderous as those in Italy were for the republican soldiers.[105]

The causes of *encombrement* in France were manifold: the high birth rate under the Empire; the end of corporate control over entrance into the medical faculties; the deeply ingrained bourgeois prejudice against mercantile activity and the post-Napoleonic decline of the army, both of which narrowed career options and operated forcefully to channel young men into what began to be called the liberal professions.

It was in the midst of this *encombrement* that a second phenomenon, medical specialization, emerged in Paris in a highly conspicuous way. Symptomatically, the year 1839 saw the founding of two new medical journals devoted to specialization, the *Revue des spécialités et des innovations médicales et chirurgicales* and *Esculape: Journal des spécialités médico-chirurgicales*. Each included the medicine of the insane within its purview and carried articles by full-fledged or peripheral members of the Esquirol circle.[106] At about the same time physicians who

[103] See George D. Sussman, "The Glut of Doctors in Mid-Nineteenth-Century France," *Comparative Studies in Society and History* 19 (1977): 287–304; and Lenore O'Boyle, "The Problem of an Excess of Educated Men in Western Europe, 1800–1850," *Journal of Modern History* 42 (1970): 471–95.

[104] See, e.g., Munaret, *Du médecin de campagne et de ses malades* (Paris: Librairie anatomique de Baillet, 1837), p. 4; and Delasiauve, *De l'organisation médicale en France sous le triple rapport de la pratique, des établissements de bienfaisance et de l'enseignement* (Paris: Masson, 1843), pp. 53–4.

[105] *La comédie humaine* (Paris: Gallimard-Pléiade, 1976–81), Vol. 8, pp. 831–32. For another contemporary fictional account of medical *encombrement*, see the popular novel by Louis Reybaud, *Jérôme Paturot à la recherche d'une position sociale* (Paris: Dubochet, Le Chevalier, 1846), p. 100.

[106] During its first year, the *Revue des spécialités* serialized a long article by Charles Londe, "De la folie: De ses causes, de ses symptômes et de son traitement." Londe, a hygienist, had "often observed *maladies mentales* with Georget and under Esquirol"; see Sachaile de la Barre, *Médecins de Paris*, p. 437. *Esculape* listed "maladies du système nerveux" as the eighth (of fifteen) specialties on its masthead. Its introductory editorial (June 1, 1839, p. 2) gave Esquirol credit for having "made a special study of mental maladies"; subsequent issues carried articles by Brierre de Boismont, Belhomme, and Moreau de Tours.

worked in lunatic institutions began, in accordance with the linguistic currents of the day, to speak of their area of medical expertise as a "specialty." (Somewhat earlier it had been referred to as a "branch" of medicine.)[107] Writing a letter of recommendation in 1845, Leuret praised a young doctor for the success with which he had plied "the study of *maladies mentale* under the direction of Monsieur Esquirol" and ended by pronouncing him "très instruit dans la spécialité."[108] Renaudin, a temporarily unemployed young physician whose only post since his graduation from the Paris faculty had been in a lunatic institution in Strasbourg, commented in 1840 on the separate identity which that medical occupation had acquired: "After having been exclusively devoted to *a specialty*, after having spent several years in an establishment for lunatics, it is difficult to give oneself over to general practice, which presents all the less chance of success because the public usually regards the doctor of lunatics as less familiar with the other branches of the healing art."[109]

Medical *encombrement* was certainly one of the factors conducing to medical specialization – that is, the latter can be regarded from one perspective as an economically rational response to the former, a subdivision of the "product" so as to increase consumer demand. This view of occupational specialization would be propounded at the end of the century by Emile Durkheim, whose *Division of Labor in Society* (1893) saw specialization as a benign solution – a "mellowed denouement" – to the Darwinian struggle for social survival. Through diversification of function, peaceful coexistence among competitors for scarce resources became possible in human society: "The oculist does not struggle with the psychiatrist (*le médecin . . . qui soigne les maladies mentales*), nor the shoemaker with the hatter."[110] Interestingly enough, ordinary observers in the first half of the nineteenth century also routinely came to the conclusion that medical specialization was a function of a competitive marketplace, of an oversupply of medical services. As early as 1828, a Paris physician suggested that, with "the number of doctors increased tenfold," only three courses of action were open to a medical man: to renounce medical practice altogether, to adopt an unscrupu-

[107] For this usage, see, e.g., Amédée Dupau in a book review in *Revue médicale historique et philosophique* 8 (1822): 347; and Rech, "Quelques considérations sur l'étude des aliénations," p. 352.

[108] AN: F^{15} 3914, letter to the minister of the interior dated September 22, 1845.

[109] Ibid., letter to the minister of the interior dated August 15, 1840, ms. p. 3, my italics. See also ADIV: X359, letter of Chambeyron to the prefect of Ille-et-Vilaine, September 26, 1840.

[110] Emile Durkheim, *The Division of Labor in Society*, trans. G. Simpson (New York: Free Press, 1933), pp. 267, 270 for "mellowed denouement"; for Durkheim's French phrase for "psychiatrist," see *De la division du travail social*, 6th ed. (Paris: F. Alcan, 1932), p. 250.

lous form of it, or to find "a specialty" and "mine it with success."[111] A German visitor to Paris, initially puzzled by the new phenomenon of medical specialization he encountered there, ascribed it to the large number of doctors in the French capital and the resultant competition among them: "The wish and the need to distinguish oneself from one's colleagues in the most remunerative way possible occasions the widespread custom of choosing one particular form, species or class of illness as one's favorite subject-matter."[112] A Paris newspaper decried "an unseemly industrialism" among doctors as revealed by the "launching of specialties."[113]

The implicit assertion here is that medical specialties had arisen in response to market conditions, that their ethos was essentially an "industrial" one. It was an assertion – and accusation – frequently made; the medical specialties, the editor of *Esculape* lamented, were "today more ill-appreciated than ever, now that they are considered as veritable *industries*."[114] Indeed, the etymology of the word "*spécialité*" bore witness to the pervasive economic interpretation of the phenomenon. The term initially arose "in our Parisian patois" in the context of mercantile competition: The shopkeeper was said to endeavor to lure the client into his shop by stocking *spécialités*, or items that could not be found elsewhere.[115] In this view – so common during the July Monarchy – that medical specialization was part and parcel of a (now-rejected) pure laissez-faire model of the medical profession, that it entailed the assimilation of the healing art to any craft or trade that had but to prove itself at the marketplace, medical specialization was once again presented as a species of charlatanism. Hence the defensive posture adopted by both of the new medical journals of the specialties; hence the warning in a popular career guide of the 1840s that if a young doctor elects to follow one of the specialties, he must be prepared to live with the "opprobrium or at least low esteem" of his colleagues.[116]

The economic roots of medical specialization, so apparent to contemporaries, cannot be discounted here. Indeed, as the correspondence of

[111] Eusèbe de Salle, as cited in a review of his *Lettre d'un médecin à un avocat*, *Journal de Paris*, April 23, 1828.

[112] Dr. Emil Kratzmann, *Die neuere Medizin in Frankreich, nach Theorie und Praxis* (Leipzig: F. A. Brockhaus, 1846), p. 191.

[113] *Correspondant*, March 24, 1847, as cited in Louis Trénard, *Salvandy en son temps, 1795–1856* (Lille: R. Giard, 1968), p. 795.

[114] June 1, 1839, p. 1, italics in original.

[115] See Georges Matoré, *Le vocabulaire et la société sous Louis-Philippe* (Geneva: Droz, 1951), p. 32; and Marc Fournier, "Les spécialités parisiennes," in *La grande ville: Nouveau tableau de Paris comique, critique et philosophique*, ed. Paul de Kock et al. (Paris: Maresq, 1844), Vol. 2, esp. pp. 57, 59: "*Spécialité*, si je ne me trompe, est née industriel" (p. 59).

[116] See editorials to the inaugural numbers of the *Revue des spécialités* and *Esculape*; the career guide is Edouard Charton, *Guide pour le choix d'un état* (Paris: Lenormant, 1842), p. 398.

Dr. Chambeyron attests, the drive to psychiatric specialization was much attenuated in areas outside Paris where the professional population was less dense and its membership less ambitious. Chambeyron had perennial trouble recruiting an intern for the Saint-Méen lunatic institution in Rennes: Sometimes he had no applicants at all; he begged the professors in the local medical school to counter the "repugnance of the students"; he lost one intern to a military hospital in the vicinity, and another was content to become an *officier de santé*, a second-class citizen of the medical professional world.[117]

Against this background, the decision of a young Parisian medical student to become a disciple of Esquirol and join the new *spécialité* becomes a complicated one. The student was probably responding, on different levels and in different degrees, to the intellectual excitement of a new paradigm; to the sentimental appeal of an ideology of humanitarianism; to Esquirol's personal magnetism as a teacher; and finally, to the economic pressures experienced by an entrant into the already overcrowded profession of medicine. But however these factors might be weighed in individual cases, serious initiation into the new specialty, and ultimately the ability to earn a living in a psychiatric career, would come via the institutional form of the circle.

[117] ADIV: Fonds Saint-Méen, letters of Chambeyron dated May 19, 1838; May 26, 1838; April 7, 1840; October 11, 1836.